A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

11 May 2026

Dr Robin Wong
Turn The Corner

Dear Dr Wong,

Re: Mr Matthew Italia
DOB: 13/12/1992 Phone: 0413 095 170
Address: 32A Hobbs Crescent, RESERVOIR, VIC, 3073
Email: matthew_italia@outlook.com

Summary

- **Small bowel Crohn's disease excluded on intestinal ultrasound.**
- **Symptomatic improvement in urgency and gut-related sensitivity on escitalopram.**
- **Continuing pelvic floor physiotherapy.**
- **No action required by you.**

I had the pleasure of reviewing Matthew via telehealth. We primarily discussed his progress since starting escitalopram and the results of his recent investigations. Although I was unable to access the formal report from St Vincent's Hospital during our consultation due to a technical issue, the clinical information suggests that the intestinal ultrasound has excluded small intestine Crohn's disease.

This finding is reassuring and allows us to focus on the management of what I suspect is a combination of gut-related sensitivity and a pelvic floor disorder. Matthew has responded well to the introduction of escitalopram, noting a reduction in the urgency and the associated panic that he had previously described. We had previously attempted to manage this with nortriptyline, which caused palpitations at higher doses, and amitriptyline, which led to excessive drowsiness. Escitalopram appears to be much better tolerated, although he did note some increased fatigue and a potential change in libido, both of which are recognised side effects.

Matthew reports that the last three weeks have been particularly stable. We discussed the physiological overlap between his mental state and gut function, specifically how stress and autonomic arousal can amplify visceral sensitivity and urgency. He has been inadvertently practicing a form of exposure therapy by managing his commute before using the toilet, which I have encouraged him to continue as it helps down-regulate these sensitive nerve pathways.

Impression and Plan

My impression is that we are successfully addressing the brain-gut axis pathways with the current combination of medication and pelvic floor physiotherapy. I have advised Matthew that we should aim for a 12-month period of symptomatic stability before any attempt is made to wean his medications, as this duration is often required to consolidate the nerve-related benefits.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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I have opted not to increase the dose of escitalopram today as he is achieving a good clinical response. He will continue his sessions with the pelvic floor physiotherapist to address the physical aspects of his bowel habit. We briefly touched on the role of his perfectionistic personality traits in driving his symptoms and the potential benefit of psychological therapy in the future; however, I do not want to overwhelm him with too many concurrent interventions. Similarly, I have advised against a dietitian review at this time to avoid further fixation on his dietary intake. I will review Matthew again in four months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Ms Isabella Lees-Trinca

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